

Influenza – Summary

Key Symptoms: Fever, headache, myalgia, malaise, cough, sore throat, nasal congestion/runny nose, etc.

Most common causing-pathogens: Influenza A & B

Non-Pharmacological Treatment: Hand washing, coughing/sneezing into sleeve, social distancing, personal protective equipment (PPE)

Pharmacological Treatment:

- **OTC:** Acetaminophen or ibuprofen for pain and fever, hydration, rest
- **Prevention:** Flu vaccine (government funded, no Rx required)
 - Recommend in all individuals > 6 months of age who have no contraindications
 - The influenza vaccine is NOT contraindicated in patients with an egg allergy
- ❖ Antiviral agents are generally not needed for healthy patients but should be used in the following populations, ideally with 48 hrs after symptom onset
 - 1-64 years of age with a chronic condition OR immunocompromised OR other risk factors
 - 65+ years of age with confirmed influenza
 - Patients who require hospitalization
- **Treatment:**
 - First Line: Oseltamivir PO BID x 5 days
 - Alternatives: Zanamivir inhaled BID x 5 days

Additional Key Points:

- In Canada, influenza season is generally considered to be November-March
- In patients < 5 yrs old and in pts > 65 years old, fever may not be present
- Renal dose adjustment is required for oseltamivir
- Zanamivir is contraindicated in those who have a milk protein allergy as well as in those with pre-existing airway diseases such as asthma & COPD
- Antivirals do not act as substitutes for the influenza vaccination in preventing influenza
- Avoid LAIV (live attenuated influenza vaccine) (intranasal) in immunocompromised children
- Children between 6 months and < 9 years of age who are receiving their influenza vaccine for the first time will require 2 doses (28 days apart). After that, they will only require 1 vaccine per flu season.